

1. CLINICAL OVERVIEW

Patient presents with **Brain** Clinical-molecular profile indicates a **MODERATE RISK** classification.

PATIENT NAME	MRN	RISK LEVEL
Sarah Jenkins	MR-2026-5541-J	MODERATE
GENDER / AGE	DIAGNOSIS DATE	WHO GRADE
female / 34	15/1/2026	II
DIAGNOSIS		
Brain		

2. PRIMARY CLINICAL RECOMMENDATION

{ "RADIATION": "RADIATION THERAPY", "SURGERY": "MAXIMAL SAFE RESECTION", "SYSTEMIC": "CHEMOTHERAPY (E.G., TEMOZOLOMIDE)", "TARGETED": "TARGETED THERAPY (E.G., IDH1 INHIBITORS) IF APPLICABLE" }

Clinical Rationale & Evidence Basis

- The patient has a localized brain tumor with an IDH1 mutation. Maximal safe resection followed by radiation therapy and chemotherapy is the standard of care for localized brain tumors. Targeted therapy may be considered for patients with IDH1 mutations.

3. GENOMIC & MOLECULAR LANDSCAPE

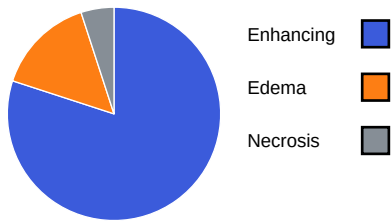
TOTAL VARIANTS	ACTIONABLE MUTATIONS
4	1

GENE	STATUS	CLINICAL IMPACT	TIER
IDH1	Mutated	Favorable prognosis	High
MGMT	Methylated	Chemo-sensitive	High
IDH1	Mutated (48%)	Pathogenic (Clonal)	High

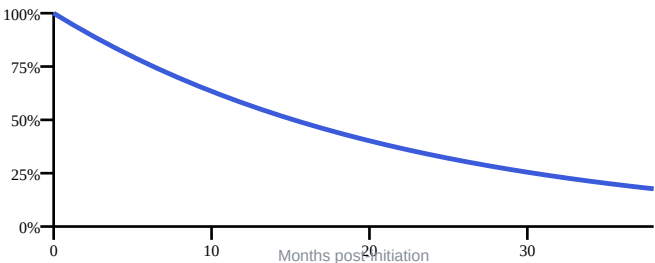
4. IMAGING & VOLUMETRIC ANALYTICS

CORE VOLUME (cc)	EDEMA VOLUME (cc)	NECROTIC %
26.58	4.98	1.66
KPS SCORE	ECOG SCORE	
100	0	

Volumetric Distribution



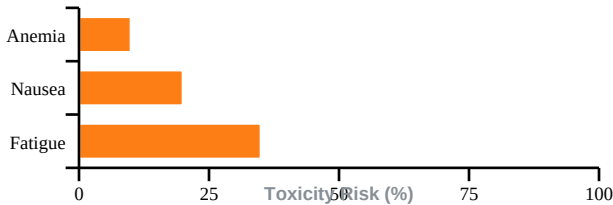
Estimated Survival Projection



5. ADVANCED OUTCOME PROJECTIONS

MEDIAN OS	MEDIAN PFS
15.2 Months	9.1 Months

Therapeutic Toxicity Profile



6. PATIENT-REPORTED OUTCOMES

30-Day Protocol Adherence: 50.0%

Recent Clinical Alerts

- [CRITICAL] CRITICAL SYMPTOM REPORTED: Fever
- [CRITICAL] CRITICAL SYMPTOM REPORTED: Fever

7. IMMEDIATE CLINICAL ACTIONS

1. Initiate recommended protocol within 7–10 days.

- 2. Schedule baseline MRI/CT for post-treatment comparison.
- 3. Monitor hematological parameters weekly if systemic therapy initiated.